

Can frameworks inform knowledge about health policy processes? Reviewing health policy papers on agenda setting and testing them against a specific priority-setting framework

Gill Walt^{1,*} and Lucy Gilson^{1,2}

¹Department of Global Health and Development, London School of Hygiene and Tropical Medicine, 15-17 Tavistock Place, London WC1 9SH, UK and ²Health Policy and Systems Division, School of Public Health and Family Medicine, University of Cape Town

*Corresponding author. London School of Hygiene and Tropical Medicine, 15-17 Tavistock Place London, WC1 9SH, UK.
E-mail: gill.walt@lshtm.ac.uk

Accepted 26 June 2014

This article systematically reviews a set of health policy papers on agenda setting and tests them against a specific priority-setting framework. The article applies the Shiffman and Smith framework in extracting and synthesizing data from an existing set of papers, purposively identified for their relevance and systematically reviewed. Its primary aim is to assess how far the component parts of the framework help to identify the factors that influence the agenda setting stage of the policy process at global and national levels. It seeks to advance the field and inform the development of theory in health policy by examining the extent to which the framework offers a useful approach for organizing and analysing data. Applying the framework retrospectively to the selected set of papers, it aims to explore influences on priority setting and to assess how far the framework might gain from further refinement or adaptation, if used prospectively. In pursuing its primary aim, the article also demonstrates how the approach of framework synthesis can be used in health policy analysis research.

Keywords National and global agenda setting, policy analysis, policy framework, priority setting, qualitative synthesis

KEY MESSAGES

- The Shiffman and Smith framework offers huge value in guiding cross-national as well as cross-policy research and analysis in a field that has been neglected and under-developed. The analysis demonstrates that comparative qualitative studies would be more rigorous if such frameworks were utilized prospectively.
- The framework would be enhanced by a few adjustments and conceptual refinements. For example, contestability or conflict is missing, and should be considered as one of the characteristics of the problem being considered. And the notion of 'guiding institutions' would benefit from being separated into two concepts: guiding organizations, under actor power, and the formal and informal norms and rules that make up judicial and legal institutions under political context.
- Framework synthesis offers a feasible, deductive approach to qualitative synthesis for health policy analysis research.

Introduction

There is a paucity of theoretical and conceptual approaches to analysis of the processes of health policy in low- and middle-income countries. This article sets out to address this neglect, and attempts to develop theory—by testing, through synthesis, a health policy framework on priority setting in order to inform future policy analysis research.

The primary aim of the article is to apply an existing framework (Shiffman and Smith 2007) in extracting and synthesizing data from a set of papers (Appendix 1) purposively identified for their relevance and systematically reviewed. It seeks to advance the field and inform the development of theory in health policy by assessing how far the different components of the framework identify factors that influence the agenda setting stage of the policy process at global and national levels. Using an existing set of papers, and applying the framework retrospectively, it aims to explore influences on priority setting and to assess how far the components of the framework might gain from further refinement or adaptation, if used prospectively. The value of analysing relevant, available papers retrospectively against the framework lies in testing how far the framework's features were reflected in papers reporting studies of agenda setting that had not been guided by this particular framework, and if factors were missing. Such framework synthesis is one of the family of approaches that can be used in qualitative review and synthesis, increasingly recognized to be of value in public health research (Barnett-Page and Thomas 2009). This approach is used here to test deductively and build existing theory, providing insights for those interested in agenda setting processes, whether policy-makers, advocates or researchers. The article demonstrates the potential and value of this approach in health policy analysis research, and is one of a set of papers reporting approaches to qualitative synthesis for this area of research (see editorial, this edition). To further develop this theoretical framework, future researchers might design a study that applies the adapted framework in studying cross-national or cross-issue agenda setting processes. Such research would also benefit from taking account of the small adaptations to the framework proposed in a set of studies published in the last few years that have explicitly applied the framework to specific issues, usually as case studies (Schmidt *et al.* 2010; Benzian *et al.* 2011; Keeling 2012; Pelletier *et al.* 2012; Tomlinson and Lund 2012).

The rest of the article is structured to provide an overview of the theoretical background to the article, a detailed outline of the steps applied in synthesis, findings from the application of the framework (combining review of relevant evidence drawn from the papers with discussion of the implications for the framework) and conclusions.

Theoretical background

The policy cycle or stages approach is one of the most enduring ways of exploring the policy process. Most scholars accept the criticism that this heuristic has major drawbacks: it appears to be overly 'rational', linear and top-down, among other things (e.g. Sabatier 2007). Its limitations, however, have not led to its abandonment, suggesting that it remains a useful heuristic to

explore a complex world. This special issue is testament to its value—and papers have been deliberately clustered around a synthesis of agenda setting, policy formulation and implementation stages of the policy process, with this article focusing on the first stage—agenda setting.

How do we assess that an issue has received global or national priority—i.e. governments are paying attention to a particular issue and have placed it on their public agenda? Kingdon (1984) argued that it was only with the confluence of three streams—problems, policies and politics—that opportunities for policy action would arise. For Baumgartner and Jones (1991), policy agendas are relatively stable, and change only when the image (the way an issue is presented or conceptualized) and the venue (the set of actors and institutions involved in a particular issue) are transformed. Stone (1989) argues that issues emerge only when people come to see them as amenable to action—and that the 'precursor' or problem definition stage to their appearance on the public agenda should not be neglected. Others have studied why some issues galvanize the attention of advocacy networks and policymakers (Hilgartner and Bosk 1988; Carpenter 2007).

All these scholars are concerned to answer the question of priority setting: given the many different problems governments face, why do some problems or issues get taken up as a priority over others? Why do some issues get on to the policy agenda, but others do not? In examining these questions, we chose to explore a specific framework on priority setting, developed by Shiffman and Smith (2007), because in comparison with other approaches it was, at the time this work began, and remains, the most developed and comprehensive-related health policy framework. We use the terms priority setting and agenda setting interchangeably in this article.

From looking at a number of empirical, country studies exploring how priority was generated for maternal health, Shiffman (2007) had earlier suggested that issues were on government agendas when the following conditions were present:

- National actors actively give attention to an issue.
- Policies addressing the issue are enacted through an authoritative decision-making process.
- Resources are provided.

The subsequent development of the framework involved adapting these conditions for global health initiatives, by including global as well as national actors actively giving attention to an issue, enacting policies to address the problem and backing up that attention with resources commensurate to the severity of the issue (Shiffman and Smith 2007). In contrast to the Kingdon and Baumgartner and Jones approaches, the Shiffman and Smith priority-setting framework (Table 1) focuses specifically on health, drawing on empirical evidence from a number of case studies to develop a generalized framework for assessing the factors that prioritize some issues over others in the process of agenda setting. Four elements describe what factors affect global and national agenda setting: the interaction between actors and their power, ideas, context of the political environment and characteristics of the issues themselves. Each of these four elements is described, and then characterized in the third column by 11 factors, such as the extent of cohesion within the policy community, as enumerated in Table 1.

Table 1 Framework for assessing what factors affect global and national agenda setting

Elements	Description	Factors shaping policy priorities
Actor power	The strength of the individuals and networks concerned with the issue	(1) Policy community cohesion: the degree of coalescence in the network involved with the issue. (2) Leadership: the presence of individuals capable of uniting the policy community, acknowledged as strong champions. (3) Guiding institutions: effectiveness of organizations or co-ordinating mechanisms. (4) Civil society mobilization: the extent to which grass-roots organizations are mobilized to support action.
Ideas	The ways in which those involved with the issue understand and portray it	(5) Internal frame: the degree to which the policy community agrees on the definition of, causes of and solutions to, the problem. (6) External frame: public portrayals of the issue in ways that resonate with external actors, especially the political leaders who control resources.
Context	The environment in which actors operate	(7) Policy windows: political moments when conditions align favourably for an issue, presenting opportunities for advocates to influence decision makers. (8) Global governance structure: the degree to which norms and institutions operating in a sector provide a platform for effective collective action.
Issue characteristics	Features of the problem	(9) Credible indicators: clear measures that show the severity of the problem, that can be used to monitor progress. (10) Severity: the size of the burden relative to others. (11) Effective interventions: the extent to which proposed means of addressing the problem are explained, cost effective, backed by scientific evidence, simple to implement and inexpensive.

Source: Shiffman and Smith (2007).

In the original article, the authors described the four elements (actor power, ideas, context and issue characteristics) as determinants of political priority for global initiatives. For this article, the focus of the framework has been interpreted to be agenda setting (which infers political prioritization) and, reflecting its antecedents, to include national as well as global action, since the categories appear to be relevant to policy agendas at both global and national levels.

The authors acknowledge limitations to the framework such as the difficulty of controlling for confounding variables, and the inevitable subjectivity in assessing the relative causal weight of the different factors. Certainly in applying the framework there was some ambiguity in assessing some of the factors: e.g. ‘guiding institutions—the effectiveness of organizations or co-ordinating mechanisms with a mandate to lead an initiative’—may have been crucial or recognizable for the issue Shiffman and Smith looked at (maternal mortality), but was less easy to judge for other policies in other contexts. However, they call for more research to help establish which factors are the most and least influential in shaping political priority.

This article partially takes up that challenge—not by undertaking new research—but by applying the framework in systematically extracting and synthesizing data from an existing set of relevant, published papers on agenda setting

(Appendix 1). This process has inevitably involved interpretation, both of the concepts within the framework, as well as in judging how far the papers reflect those factors. There are thus limitations in drawing conclusions about the framework, since the authors of the original articles did not set out to test any of its components. However, every interpretive synthesis deliberately seeks to go beyond the data of the papers reviewed within it (Barnett-Page and Thomas 2009) and the value of this article lies in its rigorous review of a set of relevant, published papers to throw light on the components included in the framework. That these papers did not use the framework initially allows the wider relevance of the framework to be considered. The main caution the authors would proffer is that the conclusions from this analysis are taken cautiously and that the framework adaptations suggested are tested empirically in prospectively designed studies. The next section describes the set of papers to which the framework was applied.

Methods: synthesizing the health policy literature on agenda setting

The following paragraphs describe the process of synthesis. This involved both authors and included engagement with the core

team involved in the broader project of qualitative synthesis for health policy analysis, and a larger group of specialists in the field of policy analysis and synthesis invited to a paper review workshop (see editorial, this edition).

Step 1: searching and identifying the literature

Identification of relevant literature began, as with other papers in this special edition (see editorial), with the body of papers identified in the first comprehensive health policy analysis literature search for low- and middle-income countries, undertaken for the period 1994–2007. As reported in Gilson and Raphaely (2008), the inclusion criteria for this review were: papers published in English; full article accessible; health policy focus; considers the process of policy change and/or factors influencing these processes; considers experience in low- and middle-income countries, including transitional countries and primary empirical study or clear empirical base. For this synthesis, those papers specifically addressing agenda setting or priority setting in health were selected from the broader pool of papers. A wider set of keywords (including initiative, politics, changing process, historical development and emergence), identified from the experience of doing the initial review, was then used in updating the review for the periods 2007 and 2009, using the same two databases as the initial review (PubMed and the International Bibliography of the Social Sciences). Finally, additional papers were identified by the team of health policy analysts involved in the broader qualitative synthesis project, each with command of the field and one with specialized knowledge of agenda setting. As noted, the fact that these papers did not use the framework initially allows the wider relevance of the framework to be tested.

Given the breadth of the field of health policy analysis, simply identifying relevant papers faced a number of challenges that need to be acknowledged. Beyond the limits of the initial review (Gilson and Raphaely 2008), it was sometimes difficult to set boundaries around the concept of agenda setting specifically (see Discussion section about distinguishing between the stages of the policy process): promising titles or abstracts sometimes turned out to be about policy formulation or advocacy. There may also have been papers addressing the concept that were not captured through titles, abstracts or keywords. However, it was known (Gilson and Raphaely 2008) that the number of papers focusing on health policy change in middle- and low-income countries was limited. In addition, the team judged that (even if not fully comprehensive) the initial set of 37 papers identified as broadly relevant to the question, but focused on an array of policies and drawn from different settings, were sufficient to demonstrate the framework synthesis approach to qualitative synthesis work.

Since the work for this article was undertaken a few additional studies have been published which applied the Shiffman and Smith framework—e.g. Schmidt *et al.* (2010); Benzian *et al.* (2011); Keeling (2012); Pelletier *et al.* (2012); Tomlinson and Lund (2012). These papers offer insights on the framework but are not included in this synthesis because they were published after the work for this article was undertaken.

Step 2: assessing the relevant literature

Each of the 37 papers initially identified through the above process was then reviewed in depth by the first author (GW), for discussion with the second author (LG). Each paper was initially examined to identify: the type of study (e.g. country case study/review), theoretical framework used (e.g. policy analysis/description), subject/theme (e.g. family planning and HIV), time period covered, authorial reflexivity (ethical and limiting considerations mentioned) and how far the policy process was described, and in particular, how far the paper was focused on agenda setting. Quality and relevance judgements were then used in the final selection of papers for inclusion in the synthesis.

Assessing the quality of the papers

Approaches to assessing the quality of qualitative research papers are recognized to take varying forms and to require judgement (Barnett-Page and Thomas 2009). Particular difficulties are faced in assessing health policy analysis work given its multiple disciplinary bases and different traditions of methods reporting (Gilson and Raphaely 2008). Rather than adopting a formal criteria-based approach or grading the papers considered, the authors drew on their judgement and experience in conducting a broad quality assessment, aiming to exclude those deemed of overall poorest quality.

Drawing on broad principles for qualitative research, quality was assessed by examining the extent to which each paper outlined a clear framework and described its methods; how far the issue was contextualized; how reflective the authors were (acknowledging weaknesses or limitations) and how far they paid attention to bias. Appendix 1 provides details on these issues for those papers finally included in the synthesis. Methods were often sparse (or missing), so it was necessary to look for clues to how the analysis had been undertaken through other comments or observations (e.g. endnotes); also judgements had to be made on theoretical frameworks or even disciplinary approaches, which were often implicit (e.g. public health framework). Journals differed greatly in their expectations of what the authors should make explicit about methods. Even if basic data were described (e.g. interviews and structured questionnaires) there was seldom anything on ‘the way’ the analysis was undertaken. Ethics approval and conflict of interest statements and a description of the contribution of each author and/or sponsors were only rarely available.

Not many authors made comments on limitations or bias. Some of the thick descriptions were written by ‘insiders’—who by dint of their institutional position or profession—had played a part in the promotion or design of specific policies. Authorial voice enriched some papers. For example, some authors appeared to be long observers of the policy process or have specialized knowledge of the issue being analysed. However, in many of the papers, authorial statements were not backed by evidence, data or examples and appeared to reflect their own experience and opinions. It was not easy to judge how far authors may have over-attributed causality to their own actions or their own institutions.

A number of other factors were considered in the quality assessment. One was the extent to which the argument was

credible and a second was the overall strength of the evidence. This was only possible to judge once the deductive process began when the papers were read with extreme care, and categorized according to themes devised within the Shiffman and Smith framework. As said, many of the papers provided little evidence to back up statements. Thirdly, this assessment of quality did not just look at the findings from papers: given the complexity and nature of the policy process, context was as important for assessing the quality of the paper, and attention was paid to how far the political and environmental context was presented in each paper.

Assessing the adequacy and relevance of the papers

The focus of the description of the agenda setting process also had to be assessed and a judgement made as to whether there was sufficient data on agenda setting to apply the framework. Some papers considered agenda setting, formulation and implementation together, some looked at agenda setting, but focused specifically on, for example, the role of the media or of evidence. One looked specifically at why an issue (abortion) was *not* on the policy agenda (Shepard and Becerra 2007). Carpenter (2007) argues that in order to understand agenda setting we have to examine issues that never made it on to policy agendas too. Focusing only on positive cases induces bias because the same factors that we think may have stood behind the rise of attention to particular issues may also have been present for issues that never received attention. A few 'advocacy papers' were therefore included as 'counterfactuals', to try to identify how far the framework factors fitted with experience around issues not yet on the agenda (Appendix 1).

Finally, given that it was not always easy to delineate the difference between the policy stages, e.g. between agenda setting and policy formulation, papers were also reviewed to see how far an authoritative decision could be discerned, and how far resources had been discussed. Shiffman (2007) suggests that the endpoint of agenda setting can be seen in three things: (1) policymakers paying serious attention to the issue, (2) the emergence of an authoritative decision and (3) allocation of resources. He also argues that priority setting, largely a public health (rather than social science) concept, is seen as part of a rational process of decision making (2008), which describes a process of deciding how to allocate limited resources (Reichenbach 2002). Although not all scholars agree that the policy process proceeds so neatly—e.g. decisions may evolve over time and it may not be easy to pinpoint an authoritative decision point (Walt 1994)—it was nevertheless deemed useful for this exercise to see how far the papers identified specific decision nodes and how far resources (in particular financial resources) were mentioned.

Step 3: final selection of papers for use in synthesis

Following assessment, the 37 papers initially identified for consideration were categorized into relevant, potentially relevant and not relevant, and overall assessments were reviewed by the second author. A key decision taken was to exclude papers published before 2000, as our initial assessment (and knowledge of the field) showed that few of these papers provided adequate data to test the framework.

Papers were, finally, included in this synthesis if they

- were empirically based, with data derived from national or observational research;
- were judged of overall adequate quality;
- focused on agenda setting or priority setting (rather than policy formulation), even if agenda setting was not explicitly referred to;
- focused on a policy relevant to and experienced in, low- and middle-income countries.

Papers were excluded if they

- focused on high-income countries (e.g. UK);
- were opinion or viewpoint papers (or judged of poor quality), or review or theoretical papers focused primarily on other stages of the policy process, such as policy formulation or legal regulation.

In reviewing relevance it became clear that there were a few papers which explicitly addressed agenda setting or priority setting, but 'prospectively'—noting issues that were neglected and needed to be raised on policy agendas [e.g. McKee *et al.* (2000) on how to get more attention for injuries among central and Eastern European governments, or Buse *et al.* (2009), which sought policy elite views on how to change policy prescriptions in order to enhance the likelihood that decision makers would be open to adopting them]. These were seen as potential 'advocacy papers', with recommendations, written purposively to promote change. They were therefore categorized separately (see Appendix 1), and the framework tested against them to see how far it might assist policymakers and others to devise strategies to get more attention for particular issues. Four papers considered relevant were in this category. The rest of the papers considered relevant fell into two categories: those that were country case studies and a few which were focused on global issues (e.g. Raviglione [2003] on how TB got on to the global policy agenda and established the global coalition against TB).

From the 37 papers initially identified, 22 were finally selected (covering 26 issues) in 3 categories, as shown in Table 2.

The 12 national case study papers selected focused on nine countries (Chile, Ghana, Peru, Kenya, South Africa, Taiwan, Thailand, India and Nepal) but covered 16 policy issues. Where one paper considered agenda setting in more than one issue (e.g. reproductive health, HIV/AIDS-related rights and sexual diversity in Peru) each issue was tested against the framework. One paper compared two countries so each country was treated separately. They fell into the following broad categories:

- National health insurance (1).
- Reproductive health/maternal health (10).
- HIV/AIDS (3).
- Vaccines (2).

Six papers covered global-level priority setting. These papers were focused on the transfer of policy between international and country jurisdictions, but focused on international organizations' policy processes. Four were largely retrospective, written by 'insiders' (e.g. staff of international or non-government organizations) whose authorial voice was judged to have

Table 2 Selected papers

Category	Focus	Number of papers
National case studies	Focused on specific middle- or low-income countries.	12
Global studies	Explored global-level agenda setting or policy transfer between global and national jurisdictions.	6
Advocacy studies	Concerned neglected issues, advocating change and strategies to get it.	4

added richness to the analysis. Three of the papers drew on interviews with global actors. They covered the following areas:

- Reproductive health (2).
- Primary Health Care (PHC) (2).
- Tuberculosis (2).

Four papers were categorized as advocacy for specific issues. The purpose of the paper seemed to be to get more attention for the issue so that it would ascend the policy agenda. It was surmised that those writing them were part of networks advocating change. Two argued that poor nutrition was a major and neglected problem in poor countries, another that policy on injuries was weak, and another interviewed policy-makers in order to scale up interventions in HIV. All four offered recommendations or strategies for change. They focused on:

- HIV/AIDS (1).
- Nutrition (2).
- Injuries (1).

Step 4: data extraction and synthesis

The framework synthesis approach was adopted in this article, entailing deductive thematic analysis of findings drawn from the included papers (Barnett-Page and Thomas 2009). In order to be able to summarize and synthesize the evidence, it was necessary to read each paper thoroughly and assess to what extent the factors in the Shiffman and Smith framework were reflected in the article.

To do this, a data extraction template was designed, based on the framework presented in Table 1. It included four sections: actor power, ideas, political context and issue characteristics. Each of these was divided into the two, three or four factors, respectively. After discussion of the first iteration of the template, the core project team advised adding a column to reflect the extent to which the issue was being seriously discussed, there were authoritative decision nodes and allocation of resources was mentioned in each paper, i.e. in order to identify whether or not an issue had reached the policy agenda. At the end of the template, a section was available for observations on issues that seemed interesting or anomalous. Data were then extracted from each paper, and entered onto an individual template.

The individual paper templates were later printed, and compared within each category (country level, global level and advocacy) and across each category. The analysis and interpretation approach was triggered by each element in the framework. It entailed iterative review of the framework element against the data extracted from the included papers, as well as comparison within and across categories of papers, to

identify whether (and how well) the data ‘fitted’ against the framework element as originally described, as well as whether there were any additional issues of relevance in agenda setting. In this exercise, the two authors’ long experience of studying health policy processes in low- and middle-income countries informed their interpretive judgements of the data from the papers and of the fit between framework element and data. Discussion of these interpretations with other project team members, participants at the 2010 draft paper review workshop and journal reviewers have allowed these judgements to be tested with others knowledgeable in the field. The final set of judgements and interpretations is presented by factor in the next section.

Testing the framework: evidence and discussion

Although the country level, global level and advocacy papers were separately reviewed, the findings from all are presented together here, noting any anomalies that were found. Each framework element is considered separately, presenting both a summary of relevant findings and broader discussion of their implications for the framework. We also added and consider a new element, outcomes, in order to consider whether the issue under consideration had reached the policy agenda.

Altogether 26 policy issues were reviewed (some papers looked at more than one issue, one paper compared two countries). Most were on reproductive or maternal health (14); four on HIV/AIDS or tuberculosis; two each on nutrition and primary health care and one each on health Insurance, vaccine introduction (in two countries) and injuries.

None of the papers were written with this particular framework in mind, and few specified what disciplines or theoretical approaches informed the analysis. Therefore, although some elements of the framework were clear and it was easy to extract data that fitted, others were more elusive.

Actor power

Actor power in the Shiffman and Smith (2007) framework is explained as ‘the strength of the individuals and organizations concerned with the issue’.

Almost all the papers centred on actors as individuals, groups or organizations, and it was easy to capture who they were, and note references to their influence or, where mentioned, their power. Authors used words such as ‘charismatic’, ‘strong relationships’, ‘insider’, ‘champion’ as well as noting positions (Deputy Minister of Health) and access to influence (close to donors).

'Coherence of the policy community' was seldom explicitly discussed among the case study papers. For example, one paper (Caceres *et al.* 2008) referred to networks of actors coming together to address a specific issue, but from different viewpoints. Many papers noted divisions in policy communities (Vianna *et al.* 2008) or limits to policy communities (Reichenbach 2002). The advocacy papers reported little policy coherence or civil society activity. They were more likely to mention adversarial competition for attention (Morris *et al.* 2008) or low priority for the specific subject (Bryce *et al.* 2008; Buse *et al.* 2009). The papers varied on their descriptions of grass roots civil society mobilization, which was strong in some case studies (Tantivess and Walt 2008), weak in others (Reichenbach 2002; Agyepong and Adjei 2008). Most papers described national bodies outside government (i.e. as part of civil society) as part of the plethora of actors, but size, representation and scale were not often explored or described.

'Leadership' in the sense of individuals able to unite the policy community, or champions of the issue, was also generally easy to identify, as were civil society organizations. Although it was not difficult to identify 'guiding institutions' (the effectiveness of organizations or co-ordinating mechanisms) the interpretation sometimes vied with the third element of the framework—political context—and in particular with 'global governance structure' (the degree to which norms and institutions provided a platform for effective collective action). In the country case studies, 'global' governance was usually translated as 'national governance', but it was not always easy to differentiate between 'guiding institutions' as actors and 'governance structure'. In Daniels and Lewin (2008) and Tantivess and Walt (2008), it was possible: the health ministries were guiding institutions in both countries, while both papers referred to national Constitutions, which could be said to fit the framework's third feature (political context), where governance (norms and institutions which provided a platform for effective collective action) was one of the factors influencing the policy agenda. The new South African Constitution recognized all citizens' right to health services and the Thai Constitution of 1997 explicitly acknowledged the right of citizens to participate in public policy making.

In several studies, the existence of the Constitution and the Courts was central to understanding the role of organized civil society groups, many of whom used such conduits to win judicial and legislative rights as well as to introduce shifts in public policies—particularly for reproductive rights over many decades (e.g. Ramasubban 2008). But the reference to the judicial system often implied more than a formalized institutional pathway: e.g. Vianna *et al.* (2008) suggested that the 1988 Constitution in Brazil reflected an end to the authoritarian regime and the re-democratization of the society.

For global studies, the notion of 'global governance structures for collective action' was difficult to fill in, and vied with guiding institutions under actor power.

Discussion

Boundary confusion is endemic in the policy literature, as the criticisms of the stages heuristic illustrate, and which makes the application of the framework difficult. In filling out the data template, it was not always obvious what data should

inhabit each cell. As discussed above, one difficulty arose regarding the difference between 'guiding institutions' (the effectiveness of organizations or co-ordinating mechanism) and 'governance' (norms and institutions that provided a platform for effective collective action). This difficulty was more acute in the national case studies where both international organizations (such as the World Health Organization) and national governments (e.g. the Ministry of Health or the President's Office or the National AIDS Council) could potentially be counted as 'guiding institutions' and also as influential in 'governance'.

The framework did not differentiate between country and global studies, where guiding institutions and governance might look different. For example, a national case study that describes its governance structure as providing opportunities for collective action would involve completely different actors from a global governance structure which purported to do the same, but was open to only a few representatives of international non-government organizations or corporate bodies. At the country level, many of the papers suggested that constitutional rights and legislative systems were particularly important in the policy process, yet this distinction was not specifically referred to in the framework. Several papers illustrated how the Constitution affected policy making. This analysis suggests that the notion of 'guiding institutions' remains under actors, but that this element is re-named 'guiding organizations' and explicitly recognized for both global and national level. Under the political context, the notion of 'governance' should be more tightly spelled out to include a definition of formal and informal institutions, i.e. a focus on formal constitutions, electoral designs and other formal institutional arrangements as well as informal—unwritten rules and practices as described by Ostrom (2007). This would give greater prominence to the role of legal and constitutional systems. An aspect of global governance that should also be explicitly included in the framework is the extent of aid or trade dependence, where external influences (e.g. from donors) on the state may affect the health policy process. This would fit under political context, and might be more relevant for some studies than others.

Finally, how far 'all' relevant actors were considered is unclear. For example, the media was seldom described as an actor, although it may be more correct to see the media under the element of ideas—shaping and reflecting societal discourse. Also while 'policymakers' were often referred to, they were more often politicians than bureaucrats or civil servants (who are acknowledged to be important players in decision-making processes), and it may be that the latter remain slightly hidden to view.

Ideas

The Shiffman and Smith (2007) framework explains ideas as 'the ways in which those involved with the issue understand and portray it'.

The ideas factor was extremely useful in identifying the way the issue is understood, and many of the papers gave historical accounts of how attitudes to controversial issues (e.g. around reproductive health) had formed and their effect on issues. Ramasubban (2008) went back to colonial history to suggest how Western homophobia and cultural inferiority invoked

during the Colonial period in India had impacted on the Penal Code introduced in 1860, and was continuing to affect attitudes to sexuality in the age of the HIV epidemic. Crichton (2008) also suggests that family planning policies (contraception) have been historically controversial, and have therefore been vulnerable to weak political commitment in Kenya (and lack of national support base). In most of the papers on sexual politics, it was possible to identify both the degree to which the policy community agrees on definition of, causes and solutions to, the problem (internal framing), and the public portrayals of the issue (external framing) that resonated with external actors, especially political leaders. In Peru, Caceres *et al.* (2008) describe the internal framing of sexual and reproductive health as dominated by a conservative Church, and with most civil society wishing to ingratiate itself with the Church, making any change difficult. Other papers showed that it was only when civil society organizations were extremely strong or had prominent champions that it was possible to frame issues about reproductive health differently.

Even where issues were not particularly controversial, framing could be an issue. Reichenbach (2002) suggests that external framing for action on breast cancer ('breast' was a powerful symbol referring to motherhood and infants) was much easier than action on cervical cancer which was portrayed in the media as associated with illicit sexual activity and poor genital hygiene. One of the questions raised by the analysis was the extent to which the framework encourages sufficient space for the active role of the media and other sources in shaping or influencing the way an issue is framed. One paper on South Africa (Jacobs and Johnson 2007) suggests that the mass media are key sites of struggle for policy issues and that there were major differences in the way the media, the state and the largest activist organization framed policies about HIV/AIDS. These sometimes differed from the external framing of the issue. It also changed over time.

Comparing the element 'ideas' with that of 'actor power' demonstrated how problematic it is to translate the concept of 'ideas' or 'framing', and how much easier it was to enumerate the actors involved in an issue. Much depended on the richness of the narrative, and how it was presented. More judgement was needed to identify both internal and external framing than where actor power lay, and different analysts might have made different interpretations. Framing, whether internal or external, was extremely influenced by contextual factors (history and culture) of the different national settings. It also provided a lead in to conflict: where issues were framed in conflicting ways, and actors adhered strongly to one particular view. The national case studies papers suggested there were several points of contestation during the policy process, but without drawing specific attention to conflict in the framework, it may be overlooked when identifying internal and external framing of ideas.

Discussion

More reflection is needed on the extent to which contentiousness or controversy should be brought out in the framework, and if so, where this notion should be logged. Although by inference, it could be noted under policy community cohesion within the actor power element, it could also come under

'ideas' or under 'issue characteristics' (because of scientific disagreement about design or interventions for instance). Conflict may also be hidden. The Peru study noted Caceres *et al.* (2008) that sexual diversity has long been of peripheral public policy interest with high levels of conflict among limited actors (Church, activists and politicians). Where common issues such as abortion, same sex practices, sex work are closely allied with public discourse on moral values, it may be a rational policy decision to underplay policy dialogue. Such policies may be strategically kept 'away from public discourse' in order to get change, and may be examples of non-decision making [see Shepard and Becerra (2007) on abortions in Chile, where there had been three failed attempts to liberalize abortion between 1989 and 2007].

In other words, traditional institutions which protect 'moral values' may work very differently in some policy areas than others, and papers may or may not capture this. Both the Brazil (Vianna *et al.* 2008) and India (Ramasubban 2008) studies support this observation. This suggests that contestation and conflict might be illustrated by examining the framing of the issue, and that it would help to identify what issue characteristics were important in assessing the likelihood of priority attention for particular issues. This would probably be more important for research design or strategic planning than for retrospective analysis.

Political context

The third factor, context, is described in the Shiffman and Smith (2007) framework as 'the environment in which actors operate'.

Again, this was clearly applicable, and there was no difficulty in recognizing policy 'windows' which offered space for advocates to introduce change or a new issue. Many of the papers mentioned opportunities that arose with a change in government, a new election, a more or less liberal country leader (Agyepong and Adjei 2008; Caceres *et al.* 2008; Daniels and Lewin 2008; Ensor *et al.* 2009), although most referred to several 'windows' over many years, suggesting that policy change is often accomplished through a process of incremental public and professional discourse (Jacobs and Johnson 2007; Munira and Fritzen 2007; Tantivess and Walt 2008).

However, as discussed above under actor power, 'global governance structure' was difficult to interpret, especially where there was overlap with 'guiding institutions'. The environment within which actors operate differs between national and global levels, and it was clear that some adjustments of interpretation had to be made depending on which level was being explored. More clarification of this element is needed as well as prospective researchers taking a more succinct approach to the notion of institutions and governance, with clear definitions of what data might be collected to represent these more abstract, but important, details.

Discussion

For this exercise, governance structures were taken to refer to *national* governance structures. However, *global* governance—norms and institutions providing a platform for effective collective action—may also be applicable for low- and middle-income countries—especially those which are

aid dependent. For example, President Bush's rulings on reproductive health (e.g. denying reproductive services to particular groups) in the early 2000s affected many countries, not only those receiving aid from the USA. For aid-dependent countries, reproductive services were limited, but middle-income countries were also affected by the discourse that resulted from Bush's policies which rejected abortion under all circumstances.

Issue characteristics

Shiffman and Smith describe issue characteristics as 'the features' of the problem. One of the features of any issue is what levels of knowledge exist about it, and what evidence there is on causation and solution; how far there are clear measures that show the severity of the problem, and how far they can be used to monitor the problem and the size of the problem in relation to others, and whether there are effective interventions. In some of the papers, these features were assumed rather than explicitly addressed, and supporting evidence was often missing. Some of the papers referred to contestation regarding both evidence and interventions between different groups—on ideological or philosophical grounds—especially on issues such as sexual or HIV/AIDS rights or on abortion. This was another example of where conflict needed to be acknowledged and it was not necessarily clear where in the framework it would sit.

The comparison between the reproductive health studies (e.g. Caceres *et al.* 2008), and, for example, the Hepatitis B vaccine study (Munira and Fritzen 2007) suggests that there may be other issue characteristics that are important: although local research to establish severity, burden and delivery feasibility was important for the introduction of the vaccine, other considerations were also central—e.g. a pharmaceutical company giving Taiwan the license to produce vaccine locally. In other words, although vaccines have sometimes been contested in particular settings, the introduction of Hepatitis B vaccine was largely uncontested in Taiwan and Thailand, once feasibility was established. However, abortion remains highly contested in some settings, and in spite of local evidence of high levels of illegal abortion, lifting a ban has been a low priority in many countries (Shepard and Becerra 2007). Crichton (2008) notes that one of the features of family planning policies is the dispersed and long-term nature of impact and lack of mobilized support from users of contraceptive services, and that these characteristics affect the way policies get on and slip off the policy agenda over time.

Discussion

We observed that finding a way to note contestability or controversy seemed to be an important element to add to the framework, although where to include it is a matter of judgement. Although it could be included under actors or under ideas—framing—it seemed to us to fit best under issue characteristics, reminding analysts that issues have particular characteristics which should be noted. In some ways, this feature of the framework allows the analyst to summarize what specific characteristics might explain the attention of a particular problem. Some issues are characterized by having multiple actors (several government departments or diverse

civil society organizations), others by having particularly powerful actors (the pharmaceutical, food, tobacco or alcohol industry); in some policy processes contestation may be muted, in others virulent; interventions may be simple or complex to deliver, needing agreement among many different departments or administrations. All these characteristics could influence the extent to which issues rise and fall on the policy agenda. Thus, we would argue that issue characteristics remains as a key feature of the framework but finding a way to include an assessment of contestation is added: e.g. whether between actors, over how issues are framed, or over doubts about evidence or interventions.

Outcome

We adapted the framework to include one final element: the *outcome*—in order to see whether the issue under discussion was being taken seriously by policymakers, was represented by an authoritative decision or had resources allocated to it. The reason for doing this was to help assess whether an issue was on the policy agenda or not, or was an issue of great interest to a few, but not being taken seriously currently by those in positions of decision making. Relevant data were not always easy to extract from the papers.

Whether policymakers were taking the issue seriously was apparent through the analysis of actor power. The evidence used to reach judgement on this feature ranged from election promises to the involvement of the President in articulating new strategy. However, the assessment depended on the issue. For example, abortion triggers intense debate, but weak policy outcomes, and several of the studies pointed to weak political positioning, especially in the face of strong or elite religious interests. In the reproductive health field especially, policy was often ambiguous, so identifying explicit nodal decisions (e.g. embodied in law or public declaration) was not always easy. Some papers referred to national policy documents, and others to parliamentary legislation, but many papers mentioned only incremental shifts in policy (e.g. to raise consciousness about breast cancer) and attempts to change the status quo that had failed.

Thus, not surprisingly given the complexity of the policy process, it was not always possible to identify an 'authoritative decision', especially where the paper reviewed policy progress over a long period. For some there were, e.g. a National Health Insurance Act passed through Parliament (Agyepong and Adjei 2008) or the integration of Hepatitis B vaccine in the national immunization programme (Munira and Fritzen 2007). For many issues, clear decision nodes were not referred to in the paper, or were ambiguous. For example, various policy documents, including national population policy, promoted rights to individuals and couples to make free, informed and responsible decisions regarding the number of children they had, but excluded abortion and sterilization (Caceres *et al.* 2008). It was not always clear, where policy documents were mentioned, whether they were translated into programmatic actions. Where papers mentioned guidelines (Daniels and Lewin 2008), this was taken as a clear indication of a decision having been made, because action would be promoted through guidelines.

However, both 'paying attention to an issue' and identifying an authoritative decision were not easy. It was apparent that

Table 3 Judging the relevance of the framework and making adjustments

Framework features	Relevance and adjustments
Actor Power—the strength of individuals and organizations concerned with the issue	Clearly relevant. ‘Guiding institutions’ overlaps with the third feature (global governance) under political context. Could be clarified by re-naming ‘guiding organizations’.
Ideas—the way in which the issue is understood	Useful for identifying how issues get framed. The role of the media in shaping or framing issues needs special attention.
Political context—the environment in which actors operate	Clearly relevant; important to include historical dimension. Governance needs to address both global and national levels; the judicial and legal institutions need to be clarified as formal and informal norms and rules (and separated from organizations and structures).
Issue characteristics	Contestability or conflict is missing from framework, and should be considered as one of the characteristics of the problem being considered.
Outcome	Helps to judge how strongly the issue is on the agenda, but each dimension needs to be further considered: e.g. allocation of resources should be broken down into financial, technical and human resources. Helpful to have analysis over a period of several decades, to tease out the main decision nodes—i.e. there is often not just one decision in the policy process that leads to change.

policymakers do not necessarily sustain interest or support for an issue over time, and may shift positions depending on other factors. South Africa’s President Mbeki framed AIDS as a disease of poverty and under-development, and, as a result, many policymakers (especially those in the Ministry of Health) were fairly mute about the epidemic during his regime. With a new President from 2009, the issue was once more taken seriously by the Ministry of Health and other public policy-makers. However, the South African case was also an example of where the state was challenged by a strong civil society movement, so that AIDS remained on the agenda, and resources flowed in from sources other than the state (Jacobs and Johnson 2007). The papers that looked at shifts in policy over decades illustrated how issues slide on and off the policy agenda (e.g. Crichton 2008).

Finally, judging what allocation of resources had been made was only occasionally explicitly mentioned. Out of the 16 national case study issues, 1 paper (Ensor *et al.* 2000) provided financing figures (e.g. the maternal incentive scheme in Nepal cost £1.15 million) and a couple of others observed (Crichton 2008) that MoH budget had contraceptive commodities added to the budget for a particular year or the MoH budget was increased specifically to fund the vaccine programme (Munira and Fritzen 2007). Several papers implied there were budgetary allocations—e.g. when family planning services were made freely available in the public service in Peru (Caceres *et al.* 2008) or when mammography machines were bought in Ghana (Reichenbach 2002), but in the majority of the papers the financing of policy decisions was not addressed explicitly, nor was there any discussion on the implications for technical, management, administrative or human resources. In only one or two cases were other resources mentioned: e.g. Ensor *et al.* (2009) specifically talked about trained health workers at deliveries.

Discussion

Adding one column on outcome to the template was helpful in judging the position of the issue on the policy agenda over time. It did not entirely resolve the question of where agenda setting ends and policy formulation begins but this may be too

elusive to pursue, given the complexity of the policy process and the time over which analysis occurs. It would, however, be useful to test the validity of adding the outcome column to the framework, with studies designed specifically to address these dimensions. In particular, there would be merit in identifying the different sorts of decision nodes (perhaps differentiating between ‘soft’ and ‘hard’ nodes—e.g. from policy statements to guidelines to statutory legislation). It would also be helpful to disaggregate the allocation of resources to get a more specific picture, by paying attention to financial, human and other resources.

Conclusions

This was an exercise in framework synthesis—an attempt to improve and advance the field of health policy analysis, which has been criticized for only weakly using theory in analysis (Gilson and Raphaely 2008; Walt *et al.* 2008). Applying the Shiffman and Smith framework was interesting and worthwhile, even as a retrospective study. Where it proved difficult to apply underlines what is already known, rather than any inherent clumsiness in the framework. The complexity of the policy process in many varied settings makes any analysis across cases difficult, and in this exercise in particular, where the cases were not designed specifically with the framework in mind.

Nonetheless, this synthesis shows that the framework adds value both to cross-country and cross-policy comparative analysis, providing a guide to what to explore in analysis of agenda-setting processes. Importantly, it has also identified some potential additions to the framework, as summarized in Table 3.

Two other observations are worth making. First, it is difficult to capture temporality in the policy process, and yet the health policy agenda does not remain static, and the decision process is influenced by past policies and positions. In synthesizing agenda-setting studies, data extraction need to be undertaken chronologically where possible, so that a picture is built up over time. This is particularly important where policies are contested and rise up and fall off the policy agenda. Capturing time

dimensions could also assist in more deliberate consideration of chains of causality among the initial four framework elements and outcomes.

Second, authorial voice needs to be considered in synthesis (perhaps as an observation under Actor Power). For example, two papers addressing a similar theme offered different versions of the policy process: one by a historian (e.g. Cueto 2004) was less rich in capturing the tensions and disagreements over time than a paper by a World Health Organization (WHO) staff member who was present during them, but wrote the paper after he had left (Litsios 2002). In contrast, the paper on tuberculosis by Raviglione (2003), a current WHO staff member, mentions little controversy or contest in the policy process, which contrasts with a paper written by academic social scientists with more interest in the policy process than in tuberculosis (Ogden *et al.* 2003). Since some of the features of the policy process are dependent on rich description rather than other sources of data, it is important to assess the legitimacy and credibility of authors in synthesizing relevant work. For example, authors who research case studies in countries which are not their own may be able to make statements a national would be loathe to make (because of political repression or career threats). On the other hand, they may misinterpret actions of actors or lack critical historical knowledge.

What are the implications for research and building the field of policy analysis?

The Shiffman and Smith framework offers huge value in guiding cross-national as well as cross-policy research and analysis in a field that has been neglected and under-developed. This article demonstrates that its use in retrospective synthesis of past health policy analysis work requires careful judgement and interpretation, and offers insights for future research.

At one level, the synthesis has generated ideas about how to adapt the existing framework in future work. At another level, the difficulties of this synthesis suggest that comparative qualitative studies would be more rigorous if frameworks such as Shiffman and Smith's were utilized prospectively. This would imply use of a comparative research design that purposively sought to assess the various elements in the framework, using the same measures for each comparative study. The synthesis raises some questions about the interpretations of commonly used concepts (governance and institutions), highlighting the need for more clarity in definition and language used. In designing cross-national or cross-policy studies, issues of interpretation and definition should be explored by research teams, so that data collected and then analysed are clearly and uniformly represented.

Finally, the framework could be applied more rigorously by focusing on similar problems and issues rather than a diversity of issues, as in this exercise. This would then allow the development of stronger propositions or hypotheses regarding the policy process, differentiating more clearly between types of policies and types of contexts. As this exercise showed, differentiating between national, global and advocacy approaches is important since there are special features to each. The empirical testing of this adapted framework, including consideration of causality, will help to develop the

field of analysis in health policy, and the authors hope that this challenge will be taken up by future researchers.

Funding

Funded by the Alliance for Health Policy and Systems Research, World Health Organization.

Conflict of interest statement. None declared.

References

- Agyepong I, Adjei S. 2008. Public social policy development and implementation: a case study of the Ghana Nat Health Ins scheme. *Health Policy and Planning* **23**: 150–60.
- Barnett-Page E, Thomas J. 2009. Methods for the synthesis of qualitative research: a critical review. *BMC Medical Research Methodology* **9**: 59.
- Baumgartner F, Jones B. 1991. Agenda dynamics and policy subsystems. *Journal of Politics* **53**: 1044–74.
- Benzian H, Hoddell M, Holmgren C *et al.* 2011. Political priority for global oral health: an analysis of reasons for international neglect. *International Dental Journal* **61**: 124–30.
- Blanc A, Tsui A. 2005. The dilemma of past success: insiders views on the future of the international family planning movement. *Studies in Family Planning* **36**: 263–76.
- Bryce J. 2008. Maternal and child undernutrition: effective action at national level. *The Lancet* **371**: 520–6.
- Buse K, Laliqi N, Mayhew SH, Imran M, Hawkes SJ. 2009. Political feasibility of scaling-up five evidence-informed HIV interventions in Pakistan: a policy analysis. *Sexual Transmission of Infections* **85**: Suppl. 11:i37–42.
- Caceres C, Cueto M, Palomino N. 2008. Policies around sexual and reproductive health and rights in Peru: conflict, biases and silence. *Global Public Health* **3**: Suppl. 239–57.
- Carpenter RC. 2007. Setting the advocacy agenda: theorizing issue emergence and non-emergence in transnational advocacy networks. *International Studies Quarterly* **51**: 99–120.
- Crichton J. 2008. Changing fortunes: an analysis of fluctuating policy space for family planning in Kenya. *Health Policy and Planning* **23**: 339–50.
- Cueto M. 2004. The origins of primary health care and selective primary health care. *American Journal of Public Health* **94**: 1864–74.
- Daniels K, Lewin S. 2008. The Practice Policy Group. 2008. Translating research into maternal health care policy: a qualitative case study of the use of evidence in policies for the treatment of eclampsia and pre-eclampsia in South Africa. *Health Research Policy & Systems* **6**: 12.
- Ensor T, Clapham S, Prasai DP. 2009. What drives health policy formulation: insights from the Nepal maternity incentive scheme. *Health Policy* **90**: 247–53.
- Gilson L, Raphaely N. 2008. The terrain of health policy analysis in low and middle income countries: a review of published literature 1994–2007. *Health Policy & Planning* **23**: 294–307.
- Hilgartner S, Bosk CL. 1988. The rise and fall of social problems: a public arenas model. *The American Journal of Sociology* **94**: 53–78.
- Jacobs S, Johnson K. 2007. Media, social movements and the state: competing images of HIV/AIDS in South Africa. *African Studies Quarterly* **9**: 127–52. [online].
- Keeling A. 2012. Using Shiffman's political priority model for future diabetes advocacy. *Diabetes Research and Clinical Practice* **95**: 299–300.

- Kingdon J. 1984. *Agendas, Alternatives and Public Policies*. Boston, MA: Little Brown.
- Litsios S. 2002. The long and difficult road to Alma-Ata: a personal reflection. *International Journal of Health Services* **32**: 709–32.
- Lush L, Walt G, Ogden J. 2003. Transferring policies for treating sexually transmitted infections: what's wrong with global guidelines? *Health Policy and Planning* **18**: 18–30.
- McKee M, Zwi A, Koupilova I, Sethi D, Leon D. 2000. Health policymaking in central and eastern Europe: lessons from the inaction on injuries? *Health Policy and Planning* **15**: 263–9.
- Morris SS, Cogill B, Uauy R. 2008. Effective international action against undernutrition: why has it proven so difficult and what can be done to accelerate progress? *The Lancet* **371**: 608–21.
- Munira SL, Fritzen SA. 2007. What influences government adoption of vaccines in developing countries? A policy process analysis. *Social Science & Medicine* **65**: 1751–64.
- Ogden J, Walt G, Lush L. 2003. The politics of 'branding' in policy transfer: the case of DOTS for tuberculosis control. *Social Science & Medicine* **57**: 179–88.
- Ostrom E. 2007. Institutional rational choice: an assessment of the institutional analysis and development framework. In: Sabatier PA (ed). *Theories of the Policy Process*. Boulder, CO: Westview Press, pp. 21–64.
- Pelletier DL, Frongillo EA, Gervais S *et al*. 2012. Nutrition agenda setting, policy formulation and implementation: lessons from the mainstreaming nutrition initiative. *Health Policy and Planning* **27**: 19–31.
- Ramasubban R. 2008. Political intersections between HIV/AIDS, sexuality and human rights: a history of resistance to the anti-sodomy law in India. *Global Public Health* **3**: Suppl. 222–38.
- Raviglione M. 2003. The TB epidemic from 1992–2002. *Tuberculosis* **83**: 4–14.
- Reichenbach L. 2002. The politics of priority setting for reproductive health: breast and cervical cancer in Ghana. *Reproductive Health Matters* **10**: 47–58.
- Schmidt M, Joosen I, Kunst AE, Klazinga NS, Stronks K. 2010. Generating political priority to tackle health disparities: a case study in the Dutch city of the Hague. *American Journal of Public Health* **100**: Suppl. 1S210–5.
- Sabatier P (ed). 2007. *Theories of the Policy Process*, Westview Press.
- Shepard BL, Becerra LC. 2007. Abortion policies and practices in Chile: ambiguities and dilemmas. *Reproductive Health Matters* **15**: 202–10.
- Shiffman J. 2007. Generating political priority for maternal mortality reduction in 5 developing countries. *American Journal of Public Health* **97**: 796–803.
- Shiffman J. 2008. Agenda setting in public health policy. *International Encyclopedia of Public Health, First Edition* **1**: 55–61.
- Shiffman J, Smith S. 2007. Generation of political priority for global health initiatives: a framework and case study of maternal mortality. *The Lancet* **370**: 1370–9.
- Stone D. 1989. Causal stories and the formation of policy agendas. *Political Science Quarterly* **104**: 281–300.
- Tantivess S, Walt G. 2008. The role of state and non-state actors in the policy process: the contribution of policy networks to the scale-up of antiretroviral therapy in Thailand. *Health Policy and Planning* **23**: 328–38.
- Tomlinson M, Lund C. 2012. Why does mental health not get the attention it deserves? An application of the Shiffman and Smith framework. *PLOS Medicine* **9**: e1001178.
- Vianna ARB, Carrara S, Lacerda P. 2008. Sexual politics and sexual rights in Brazil: an overview. *Global Public Health* **3**: Suppl. 25–21.
- Walt G. 1994. *Health Policy: An Introduction to Process and Power*. London: Zed Books.
- Walt G, Shiffman J, Schneider H *et al*. 2008. Doing health policy analysis: methodological and conceptual reflections and challenges. *Health Policy & Planning* **23**: 308–17.

Appendix 1

Assessment of selected papers

Table A1 National level

Author, year, title and journal	Methods (framework data collection and analysis and credible argument)	Research question and context (adequately described)	Reflexivity (consideration of weaknesses and ethics)	Bias (of authors or institutions)
1. Agyepong I, Adjei S. 2008. Public social policy development and implementation: a case study of the Ghana National Health Insurance Scheme. <i>Health Policy and Planning</i> 23 : 150–60.	Policy analysis framework: actors, context, and processes, and effects policy and programme content. Qualitative case study Participant observation, review of media and public sector historical and policy documents.	Describes and reflects on challenges of introduction of universal health insurance in Ghana. Context includes socio-economic and demographic factors; health status, access and political and historical context of health insurance. Provide 'thick' description.	Analysis relies on authors' observations and participation in public sector over a decade. Paper tries not to pass value judgments on success/failure. Nothing on ethics.	Insider knowledge. Both authors part of design and implementation of insurance system. Provide endnote giving professional history of each author so readers can judge bias.
2. Caceres C, Cueto M, Palomino N. 2008. Policies around sexual and reproductive health and rights in Peru: conflict, biases and silence. <i>Global Public Health</i> 3 (Suppl. 2): 39–57	Framework: social science and public policy perspective (not directly policy process). Qualitative case study of Peru. Data from secondary data sources, e.g. reports, legal texts, media, policy documents and academic papers, plus eight key informant interviews (not specified by individual).	Examination of shifts in government policy with regard to reproductive rights, HIV/AIDS prevention and treatment and sexual diversity. Context and actors described historically over 30 years, with focus from 1990.	Nothing on limitations. Reflective discussion on politics of state and other actors, and how they affect policies.	Based on longer study undertaken for Ford Foundation Chapter in e-book <i>Sex Politics</i> by R Parker <i>et al</i> (2007). Authors all at School of Public Health, Cayetano Heredia University; Lima, but no biographies. Academic insiders?
3. Crichton J. 2008. Changing fortunes: an analysis of fluctuating policy space for family planning in Kenya. <i>Health Policy & Planning</i> 23 : 339–50	Framework: 'Policy space' analysis: context, decision-making circumstances and policy characteristics. Case study Kenya: 13 semi-structured interviews, and 3 unstructured discussions and documents, official and grey. Textual analysis and triangulation of data with key informants. Where discrepancies or information gaps were found, further investigation done by telephone or looking at grey literature.	Explores dynamics of routine policy and programme evolution and challenge of sustaining support for issues on policy agenda. Context: section specifically covering contextual factors, political, economic and social over a decade.	Acknowledges case study approach has limitations with regard to causality, but still supports lessons on policy processes for other settings.	Funded by DFID; author based in Kenya during research for Realizing Rights Research Programme Consortium. Academic outsider?
4. Jacobs S, Johnson K. 2007. Media, social movements and the state: competing images of HIV/AIDS in South Africa. <i>African Studies Quarterly</i> 9 : 127–52	Framework uses discourse analysis by focusing on the main frames used by the media through which AIDS discourse was constructed. Concept of frames derived from Goffman. Do not provide more on methods, or how frames were analysed.	Explore central role of South Africa's media in shaping the discourse about HIV/AIDS Context focuses on President Mbeki, Treatment Action Campaign (TAC) and media	Nothing on limitations	Biographies suggest both authors in political science. Jacobs at University of Michigan, Johnson at Agnes Scott College, GA, USA. She has worked in southern Africa for a decade. Outsider academics?

(continued)

Table A1 Continued

Author, year, title and journal	Methods (framework data collection and analysis and credible argument)	Research question and context (adequately described)	Reflexivity (consideration of weaknesses and ethics)	Bias (of authors or institutions)
5. Daniels K and Lewin S; The Practice Policy Group. 2008. Translating research into maternal health care policy: a qualitative case study of the use of evidence in policies for the treatment of eclampsia and pre-eclampsia in South Africa. <i>Health Research Policy & Systems</i> 6 : 12.	Research utilization framework: ideas, interests, institutions plus policy process and agenda setting. Qualitative case study, SA, literature and policy documents review, events timeline and 15 interviews. Data analysed thematically—latent and manifest themes.	How research information informed policy making and clinical guideline development in use of magnesium sulphate for eclampsia. Context sketched briefly.	Section on limitations of study covers four points demonstrating reflexivity of authors and process of research. Ethics approval given by various bodies; also consent forms for interviews.	Explicitly describe how two authors shared analysis of data. Research funded by Alliance for Health Policy and Systems Research; German development agency (GTZ) and European Union (EU) among others. Insider/outsider academics.
6. Munira SL, Fritzen SA. 2007. What influences government adoption of vaccines in developing countries? A policy process analysis. <i>Social Science & Medicine</i> 65 : 1751–64.	Framework: policy process approach: policy characteristics, health system characteristics, actors, process and context. Comparative case study on Taiwan and Thailand. Literature Review on vaccine uptake and adoption in these two countries to provide descriptive account of uptake. Compare and contrast these accounts with nine expert interviewees in person and by email.	What combination of factors led to uptake of Hepatitis B vaccine in these two countries.	Noted shortcomings in literature review which did not consider 'causal complexity' and neglected policy process and context. Under conclusions discuss need for different methodologies to indicate causal pathways (p. 1782).	Disclaimer from one author at GFATM; the other at School Public Policy, National University of Singapore. Outsider academics.
7. Ramasubban R. 2008. Political intersections between HIV/AIDS, sexuality and human rights: a history of resistance to the anti-sodomy law in India. <i>Global Public Health</i> , 3 (Suppl. 2): 22–38	Framework: cultural, economic and political factors. Case study, India. No methods described: in acknowledgements say mix of primary and secondary document sources and in-depth interviews used.	Identifies cultural, economic and political forces acting on the evolution of a sexual rights' agenda in India. Context: described as above.	Nothing noted.	Part of longer study funded by Ford Foundation. Chapter in e-book <i>Sex Politics</i> by R Parker <i>et al.</i> (2007). Insider academic? (Centre for Social and Technological Change, Mumbai).
8. Reichenbach L. 2002. The politics of priority setting for reproductive health: breast and cervical cancer in Ghana. <i>Reproductive Health Matters</i> 10 : 47–58.	Framework of priority setting, which includes quantitative measures, e.g. DALYS; and qualitative political, social and organizational factors to distinguish between normative aspects of priority setting and what actually happens. Case study Ghana. Methods include 115 key informant interviews; documents and secondary data and review of parliamentary proceedings. Content analysis of interviews and documents and comparison between breast and cervical cancer.	Examines how political factors gave higher priority to one cancer over the other, in spite of scientific and economic evidence to the contrary. Context discussed in relation to breast cancer and cervical cancer over period 1990–97.	No discussion of weaknesses or limitations, although does have cautionary point in relation to interpretation of evidence, i.e. evidence is important but not always predictable.	Author had institutional affiliation to Ghana School Public Health. Paper based on doctoral dissertation, with funding from Hewlett Foundation and a travel grant. Outsider academic

(continued)

Table A1 Continued

Author, year, title and journal	Methods (framework data collection and analysis and credible argument)	Research question and context (adequately described)	Reflexivity (consideration of weaknesses and ethics)	Bias (of authors or institutions)
9. Tantivess S, Walt G. 2008. The role of state and non-state actors in the policy process: the contribution of policy networks to the scale-up of antiretroviral therapy in Thailand. <i>Health Policy & Planning</i> 23 : 1–11.	Policy process framework, looking at agenda setting, formulation and implementation. Case study Thailand 80 key informant interviews, documents. Direct observation, with triangulation. Framework approach used for analysis identifying themes and patterns.	Explores the role of national policy networks in ART policy development and local network involvement in policy implementation. Context noted in terms of shift to involve non-state actors in state decision making.	No weaknesses or limitations mentioned, although 'insider knowledge' and participation in policy process is noted as an implied advantage.	Draws on doctoral research by one author. Insider policymaker/academic.
10. Vianna ARB, Carrara S, Lacerda P. 2008. Sexual politics and sexual rights in Brazil: an overview. <i>Global Public Health</i> 3 (Suppl 2) 5–21.	Framework: political construction of sexual rights. Case study Brazil No methods described.	Surveys political construction of sexual rights, maps actors, legal instruments, public policies over two decades in Brazil. Context of public policies and rights, from 1988 Constitution.	None mentioned	Authors at Federal University of Rio de Janeiro, Latin American Center on Sexuality and Human Rights. (Departments of anthropology, social medicine, and sexuality and human rights). Part of longer study funded by Ford Foundation. Chapter in e-book <i>Sex Politics</i> by R Parker <i>et al.</i> (2007).
11. Ensor T, Clapham S, Prasai DP. 2000. What drives health policy formulation: insights from the Nepal maternity incentive scheme. <i>Health Policy</i> 90 : 247–53.	Framework: role of research in policy development. Case study Nepal Analysis based on results from larger study, but no discussion on how it was undertaken.	Explores reasons for Government acceptance of a policy to overcome financial barriers to access to maternity care. Context: very brief focusing on safe delivery incentive programme.	Note that researchers can adapt process of research to increase likelihood of policy impact but cannot guarantee acceptance.	Researchers involved in larger study, from Nepal or Aberdeen. DFID funded Outsider academic, inside researcher.
12. Shepard BL, Becerra LC. 2007. Abortion policies and practices in Chile: ambiguities and dilemmas. <i>Reproductive Health Matters</i> 15 : 202–10.	Framework not explicit (public health) Case study Chile Methods unclear, but draw on experience, literature review and interviews from larger study on abortion undertaken by Shepard.	Explore how abortion law results in dilemmas for women and professionals. Context: brief overview of political and legal developments.	None mentioned	Ford Foundation funded study by Shepard. Becerra is researcher and professor of law at the University of Diego Portales, Santiago, Chile. Outsider/insider academics.

Table A2 Global level

Author, year and journal	Methods (framework data collection and analysis and credible argument)	Research question and context (adequately described)	Reflexivity (consideration of weaknesses and ethics)	Bias (of authors or institutions)
13. Blanc AK, Tsui A. 2005. The dilemma of past success: insiders' views on future of international family planning movement. <i>Studies in Family Planning</i> 36 : 263–76.	Framework: historical overview of social movement; use Mauss five stages as socio-logical framework. Methods include 27 key informant interviews, two focus groups with 'insiders'.	Examine factors contributing to declining international visibility of the family planning movement. Draw on social movement, globalization and transnational networks literature.	Describe consideration given on 'who' to sample for interviews. Promised confidentiality to those interviewed and in focus groups.	Both authors involved in reproductive health and staff of foundations. Tsui also has appointment at Johns Hopkins School of Public Health. Insider academics.
14. Cueto M. 2004. The origins of primary health care and selective primary health care. <i>American Journal of Public Health</i> 94 : 1864–74.	Framework: historical study looking at context, actors, targets set and techniques proposed. Methods used contemporary documents and library and archive sources, a few interviews; examples from Latin America.	How did Primary Health Care originate and what led to development of alternative, selective primary health care.	None	Wrote paper while visiting fellow at Woodrow Wilson Center, Washington. Funded by Fulbright and Joint Learning Initiative. Is based at Heredia University, Peru. Academic historian.
15. Litsios S. 2002. The long and difficult road to Alma Ata: a personal reflection. <i>International Journal of Health Services</i> 32 : 709–32.	Framework: implicit, public health. Methods based on recollection, historical documents and records from WHO; focus on 1970–78 on decision making in WHO.	Asks questions in relation to why initially Alma Ata conference was wanted by Soviets and not WHO, and what would have happened if it did not take place. Context only in relation to WHO and Soviet Union and discussion around PHC.	None	Litsios was staff member at WHO at the time; not clear when he left. Insider policymaker/analyst.
16. Ogden J, Walt G, Lush L. 2003. The politics of branding in policy transfer: the case of DOTS for tuberculosis control. <i>Social Science & Medicine</i> 57 : 179–88.	Framework: includes policy transfer mechanisms and Kingdon's agenda setting factors. Methods: 40 interviews, literature review and review of documents.	Demonstrates how resources mobilized to put tuberculosis on national and international policy agendas, and how policy branded as Directly Observed Treatment, Shortcourse (DOTS) was transferred to low-income countries.	None mentioned	All authors at LSHTM at the time. Funding from ESRC and some from DFID. Academic social scientists/researchers.
17. Lush L, Walt G, Ogden J. 2003. Transferring policies for treating sexually transmitted infections: what's wrong with global guidelines? <i>Health Policy & Planning</i> 18 : 18–30.	Policy analysis framework, using agenda setting and policy transfer. Methods: 40 interviews, documents and literature review.	Explores how Sexually Transmitted infections (STIs) rose on the policy agenda and how guidelines for syndromic management were disseminated to low-income countries.	None mentioned	ESRC funding, some DFID. All authors at LSHTM at time. Academic social scientist/researchers.
18. Raviglione M. 2003. The TB epidemic from 1992 to 2002. <i>Tuberculosis</i> 83 : 4–14.	Framework: implicit public health. No methods, but 'insider' view.	Review of TB control in early 1990s, and how international support for TB changed. Context: international in relation to TB.	None mentioned	Staff member of WHO TB programme. Insider policymaker.

Table A3 Advocacy papers (Counterfactuals—issues *not* on policy agendas)

Author, year and journal	Methods (framework data collection and analysis and credible argument)	Research question and context (adequately described)	Reflexivity (consideration of weaknesses and ethics)	Bias (of authors or institutions)
19. Buse K, Laliji N, Mayhew SH, Imran M, Hawkes SJ. 2009. Political feasibility of scaling-up five evidence-informed HIV interventions in Pakistan: a policy analysis. <i>Sexually Transmitted Infections</i> 85 : (Suppl. 11): ii37–42.	Framework: implicit public health. Case study in Pakistan; 40 questionnaires emailed (22 completed); 12 interviews.	Explores perceptions on feasibility of introducing five different HIV interventions, suggests strategies to overcome problems. Context of HIV/AIDS in Pakistan described briefly.	Has section on limitations, e.g. very limited number of purposively selected informants, etc. Endnote describes what each author contributed.	Authors at UNAIDS and National AIDS Control programme and LSHTM. Outsider/insider academics and policymakers.
20. Bryce J. 2008. Maternal and child undernutrition: effective action at national level. <i>The Lancet</i> 371 : 520–6.	Framework: not explicit—public health. Part of larger study, consisting of systematic review of papers with nutrition component; structured questionnaire to UNICEF and WHO staff in 20 countries; 30 interviews from practitioners in 12 countries.	Seeks to define strategies for improving maternal and child undernutrition. Identifies challenges for addressing undernutrition and advocacy for international and national systems	Who contributed what is described. One author acknowledges potential conflict of interest—staff at UNICEF.	Maternal and Child Undernutrition Study Group. Funded by Gates Foundation, with participation of WHO and UNICEF. Insider policy community: academic, policymakers, sponsors.
21. Morris SS, Cogill B, Uauy R. 2008. Effective international action against undernutrition: why has it proven so difficult and what can be done to accelerate progress? <i>The Lancet</i> 371 : 608–21.	Systems framework: stewardship, mobilization of resources, direct provision of nutrition services and human and institutional strengthening. Multiple methods: systematic review; quantitative analysis. Financial flows, semi-structured and key informant interviews, problem tree analysis focusing on 15 key organizations.	Review data to assess performance in each area and make recommendations to overcome some important problems. Also propose five priority actions for development of new international architecture for nutrition.	‘We should acknowledge that the analyses and recommendations ... necessarily reflect the organizational experience and disciplinary biases of the authors, and others might have emphasized differently the various strands of data that we present ...’ State who contributed what.	Members of Maternal and Child Undernutrition Study Group. Funded by Gates Foundation (say Gates not involved in data collection, interpretation, etc.) though attended meetings and discussed progress of paper. Insider policy community.
22. McKee M, Zwi A, Koupilova I, Sethi D, Leon D. 2000. Health policy-making in central and eastern Europe: lessons from the inaction on injuries? <i>Health Policy & Planning</i> 15 : 263–9.	Framework: identify factors in policy agenda that inhibit action and propose set of prerequisites for effective policies: visibility, capacity, ownership, intersectoral action and effective government. Methods not described—prerequisites drawn on experience of understanding policy agenda.	Seeks to identify factors that have inhibited policy development on injuries and to draw lessons more generally. Context briefly covered under each prerequisite.	Author biographies give some information on particular knowledge or experience each brought to the analysis.	Authors all researchers at LSHTM at the time. Academic researchers with different disciplinary foci.